

Medicare and Medicaid Billing Guidelines

Sunrise Medical Center

Document Title: Medicare and Medicaid Billing Guidelines

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Department: Patient Financial Services

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Distribution: All Sunrise Medical Center Staff

1. Purpose

This guideline standardizes the billing, coding, and documentation practices required for claims submitted to Medicare and Medicaid on behalf of patients treated at Sunrise Medical Center. Accurate and compliant billing protects the hospital from audit findings, recoupment demands, and False Claims Act exposure.

2. Medical Necessity Documentation

Every billed service must be supported by clinical documentation establishing medical necessity at the time the order was written, not reconstructed after the fact. Physicians and advanced practice providers must document the specific clinical indication for tests, procedures, and extended length of stay in terms that map directly to the diagnosis codes submitted on the claim.

3. Inpatient vs. Observation Status

Patient status determination (inpatient versus outpatient observation) must be made using the current CMS Two-Midnight Rule and documented by the admitting physician, with Utilization Review confirming status within 24 hours of admission. Incorrect status determination is one of the most frequent sources of Medicare Recovery Audit Contractor (RAC) findings and must be escalated to Utilization Review immediately if uncertain.

4. Coding Standards

All diagnosis and procedure coding is performed by certified coders using ICD-10-CM and ICD-10-PCS, referencing the complete medical record rather than the discharge summary alone. Coders must query the physician using the standard Coding Query Form whenever documentation is ambiguous, incomplete, or conflicting — coders may never assign a code based on assumption.

- Upcoding (billing for a higher level of service than documented) is strictly prohibited and subject to immediate compliance investigation.
- Unbundling (billing separately for services that should be billed as a single bundled code) is prohibited absent a valid modifier and documentation.
- All present-on-admission indicators must be assigned accurately, as they directly affect Medicare Hospital-Acquired Condition payment adjustments.

5. Secondary Payer and Coordination of Benefits

Registration staff must verify Medicare Secondary Payer status at every admission, including questions about employer coverage, workers' compensation, auto liability, and other insurance that may be primary to Medicare. Failure to properly coordinate benefits can result in mandatory repayment with penalties.

6. Credit Balances and Refunds

Identified overpayments from Medicare or Medicaid must be reported and refunded within 60 days of identification, per the Affordable Care Act's 60-Day Rule. Patient Financial Services runs a monthly credit balance report and any government payer credit is escalated to the Compliance Office for review prior to refund processing.

7. Audit Response Procedure

All Medicare and Medicaid audit requests (RAC, MAC, ZPIC, or state Medicaid program integrity) must be forwarded immediately to the Compliance Office and Patient Financial Services Director. No records may be released, and no informal response provided, without Compliance Office review, even for routine-seeming documentation requests.

8. Related Documents

SMC-FIN-045 Charge Description Master Maintenance Policy; SMC-COMP-022 False Claims Act and Fraud Prevention Policy; SMC-UR-003 Utilization Review and Two-Midnight Rule Procedure.

Appendix A: Frequently Asked Questions

Q: What should a coder do if physician documentation is unclear about a diagnosis? A: Submit a formal Coding Query Form to the physician requesting clarification; the coder cannot assign a code based on inference and must wait for a documented physician response.

Q: Who decides whether a patient is inpatient or observation status? A: The admitting physician makes the initial determination based on expected length of stay and clinical severity, with Utilization Review confirming appropriateness within 24 hours using CMS criteria.

Q: What happens if an overpayment from Medicaid is discovered six months after the claim was paid? A: It must still be reported and refunded within 60 days of discovery, regardless of how much time has passed since the original payment, per the 60-Day Rule.

Appendix B: Common Scenarios

Scenario — Patient status changes mid-stay: A patient initially placed in observation status develops complications requiring a longer stay. Utilization Review must re-evaluate and, if criteria are met, formally convert the patient to inpatient status with physician documentation supporting the change.

Scenario — Auditor requests records for a five-year-old claim: All requests are routed through the Compliance Office regardless of claim age; records retention policy ensures documentation remains available for the statutory audit lookback period.

Revision History

Version	Date	Description
1.0	Feb 1, 2022	Initial release
1.2	Nov 3, 2025	Updated 60-Day Rule refund process and RAC escalation path